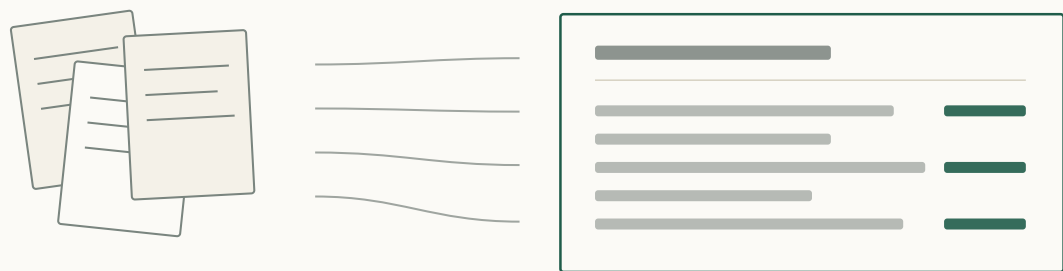


# One view of every site.

Every practice in your group has a back room of paper and no consolidated view across any of it. We turn that archive into searchable, structured patient data — site by site, on a predictable monthly cost, without a group-wide system rollout.



Built for South African private practice — medical schemes, ICD-10, NAPPI, ZAR billing. Not an international system bent to fit.

**SEE IT FIRST**  
ON ONE OF YOUR SITES'  
OWN FILES. TWENTY MINUTES.

We don't know which of these is the pressure on your desk this quarter — you do. We've written to the four we hear most from people running a group of practices, so you can find the one that is actually yours rather than the one we guessed.

#### Cost you can't see whole

Each site's admin burden and leakage is real, but the group-level number lives in nobody's hands — it's reconstructed by ringing each practice manager.

#### Exposure across sites

A scheme audit or a clinical query lands on one site; the answer is in a filing cabinet in a different building. The group carries that risk, not the practice.

#### No consolidated view

You cannot answer a basic clinical or financial question across your own group without a round of phone calls. The data exists; it isn't one thing.

#### Growth friction

Every practice you bring in arrives as another back room of paper in another filing system. Integration is a tax on the growth strategy itself.

Whichever of those is yours, it sits on the same foundation.

Every one of those pressures has the same thing underneath it: each site has years of patient files that are paper, and the group has no structured, searchable view across any of them. You cannot consolidate cost, defend an audit across sites, see the group whole, or absorb an acquisition cleanly while the underlying record is a building full of folders. The foundation is the same regardless of which pressure is loudest — so that is where we start, and it is the only thing we ask you to start with.

The same thing at every site, so the group result is consistent rather than four practices each solving it differently — which is itself part of the problem you have today.

## 1 Each site's archive becomes structured data — without a rollout.

A practice scans into the folder it already uses. The records come back read, structured, and checked by that site's own staff in about 30–60 seconds per document. No new system imposed on every site at once, no group-wide migration project, no months of admin. One site can start without waiting for the others.

## 2 The group becomes answerable, not a round of phone calls.

Once a site's records are structured, a question can be asked across them and answered with the original scanned page one click away. As more sites come on, the same questions extend across the group — the consolidated view is a consequence of the digitisation, not a separate system you also have to buy and roll out.

## 3 The audit answer stops living in a different building.

Every approval and correction on a patient record is logged — who did it, when. When a scheme or an inspector asks about a patient at any site, the basis of the record is a query, not a week of someone driving between practices with a box of folders.

You've almost certainly been pitched a group-wide system before. The three questions a group actually asks, answered straight.

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*"Is this another rollout my sites will resist and that runs over?"*

**No site has to change how it works.** Reception scans into the folder it already uses; the clinical workflow is untouched. Sites adopt one at a time, at the pace you choose — there is no flag day, no group-wide cutover, and a site that starts late loses nothing.

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*"What does this cost across the whole group, really?"*

**It is a predictable monthly subscription per site, not a capital project.** There is no six-figure group digitisation quote to take to a board. Cost scales with sites and volume, it is opex not capex, and a site can be added or stopped without unwinding the others. The specific numbers are a conversation, not a figure on a cold document.

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*"Could the AI be confidently wrong, across all my sites at once?"*

**The system extracts; a person at that site approves.** Nothing enters a patient record until a human confirms the right patient and the right data. The safety gate and the human-approval step are how it is built underneath — the people who built it have shipped clinical software before and treat the first wrong-patient record as the moment a group stops trusting a vendor forever.

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We will not ask you to take any of that on faith. The next page is the whole offer, and it is deliberately small.

One thing first, at one site. Not a group programme. The foundation every one of your pressures sits on, proven on real files before it goes anywhere near the rest of the group.

**Digitise one site's archive. That is the whole first commitment.**

Not the group. Not analytics. Not a platform. One site's back room becomes structured data on a monthly subscription — a real, complete result on its own, even if you never do anything else with us.

**See it work on that site's own files first.**

A short session on the messiest archive that site has. You see what comes out before any commitment widens beyond it.

**Extend site by site, on your timing.**

Each further site is the same predictable step. The group result accrues as sites come on — you are never running a group-wide project, only repeating one proven small one.

WHERE THIS GOES — LATER, AND ONLY IF YOU CHOOSE

Once sites are digitised, the structured data is what makes a cross-site view, group analytics, and the wider platform possible — each depends on the digitisation having happened first; none of it works on a filing cabinet. That is a later conversation on your timing, named here so it is visible, not promised here as part of what you would be buying. What you would be buying is the digitisation. Everything else is what that earns.

WHERE WE'RE STRAIGHT WITH YOU

Our reference today is a single, early one: a two-doctor practice, open since 2010, with roughly 20,000 patient files at about ten pages each. The standard once-off digitisation quotes they were given ran to around R400,000 — the kind of project bill that gets shelved. On a monthly subscription, that same archive becomes structured data at a predictable cost, site by site. We are telling you plainly that this is one practice, not a group track record — because the mechanism that worked there is the *same* per-site step repeated across a group, and we would rather you saw exactly what we have than be shown more than we have.

THE ONLY THING WE'RE ASKING FOR

## Pick your worst site's worst shelf.

Choose the practice in the group whose archive you would least want to digitise by hand, and the messiest shelf in it. We'll show you, on those real files, in about twenty minutes, exactly what comes out the other side. Then you decide whether it goes wider.

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Progno Labs

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